Appendix C

Non-traumatic Spinal Cord Injury Pathway

Northern Devon Healthcare NHS
NHS Trust
Incorporating community services in Exeter, East and Mid Devon

Inclusion Criteria

- Clinical diagnosis of spinal cord injury resulting in full or partial, para or tetraplegia as a result of non-progressive non-traumatic insult
- Malignant spinal cord compression

Exclusion Criteria

- Injury to the vertebral column without cord

Adult presents with acute presentation of non-traumatic Spinal cord injury (SCI)

Emergency Department | **Inpatient**

Is there a clinical diagnosis of malignant spinal cord compression?

YES → **Refer to Medics**

NO → **Refer to Trauma & Orthopaedics**

Assessment & Clinical Diagnosis of Non-Traumatic SCI

Completed by Consultant-led Orthopaedic Team

Primary Survey confirms possible spinal cord injury
ASIA completed
CT Scan of affected area of spine OR Trauma CT scan according to CT protocol
Non-contiguous injury must be excluded

Adult with SCI repatriated to NDDH

Initiate SCI Care Pathway

Contact Spinal Consultant at RD&E/ Derriford/Salisbury

Non-Traumatic: call within 24 hours of confirmed clinical diagnosis of SCI

- *Consultant/Registrar to Spinal Consultant telephone discussion.*
- *Please have Medical notes, Observation and Medication Charts to hand.*
- *Ensure assessments completed, including ABCDE assessment as per ATLS guidelines, organ support level, ASIA findings, injury summary and current management.*
- *Appropriate location for optimum management agreed (including surgery).*
- *Document variance if telephone referral outside the above time limits.*

Orthopaedic doctor to document patient's management plan in SCI pathway

Transfer to Capener Ward
(unless for transfer to RD&E/Derriford or primary care needs require patient be on alternative ward)*

**Contact Clinical Site (bleep 500) to instigate Capener outreach/backfill plan*

Ward staff to follow Spinal Management Plan & commence relevant care plans
(e.g. Multi-Professional SCI / Aspen Collar / Spinal Brace)

If patient discharged home with a collar/brace, follow 'Collar SOP'